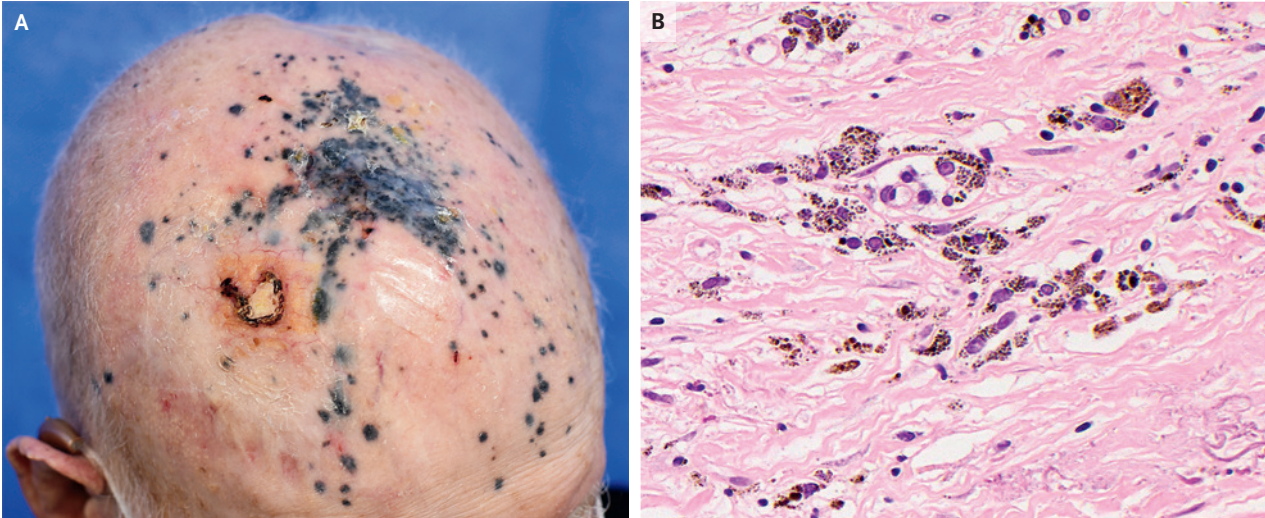


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Tumoral Melanosis



Deshan F. Sebaratnam, F.A.C.D.,¹
and James P. Pham, M.D.¹
¹Liverpool Hospital, Liverpool, NSW,
Australia.

Dr. Sebaratnam can be contacted at
deshan.sebaratnam@health.nsw.gov.au.

AN 84-YEAR-OLD MAN WITH A HISTORY OF MELANOMA OF THE RIGHT forehead complicated by in-transit metastases to the scalp presented to the dermatology clinic. Seven months before the current presentation, treatment with two immunotherapy agents had been started, which had resulted in shrinking of the metastatic scalp nodules. The nodular metastatic deposits were then replaced by gray spots that had emerged across his scalp. On physical examination, there were multiple blue-gray macules on the scalp, as well as an unhealed ulcer from trauma (Panel A). Dermoscopy-guided biopsy of four macules revealed scattered melanin-laden macrophages within the superficial and middle dermis without any evidence of residual melanoma (Panel B). Immunohistochemical staining of a biopsy sample for markers of melanoma was also negative. A diagnosis of tumoral melanosis was made. Tumoral melanosis is a pigmented skin lesion that results when melanin released from melanoma cells is taken up by dermal macrophages. It can be seen after regression of melanoma, sometimes as a response to immunotherapy. Absence of nodularity is supportive of tumoral melanosis; however, histologic assessment is needed to differentiate melanoma metastases from tumoral melanosis. Patients with tumoral melanosis should undergo regular dermatologic follow-up, with any new lesions — particularly papules or nodules — to be considered for biopsy. Ongoing surveillance for recurrent melanoma was continued in this patient.

DOI: 10.1056/NEJMicm2413467

Copyright © 2025 Massachusetts Medical Society.